

in climate funding.¹¹ For example, only 2.0% of global funding for climate adaptation and 0.5% of overall climate funding is designated to addressing health outcomes within the context of climate change.¹⁰ As the world's largest economy, the USA has the opportunity to transform its own contribution and to incentivise other nations to do the same. Equally, the USA can help guide the direction of global funding. Priorities should include, for example, strengthening a well distributed, skilled, fit-for-purpose workforce that is crucial for disease prevention, early warning, diagnosis, treatment, and care, as well as strengthening the early warning system for health threats tied to climate (panel). The USA can reinvigorate its 2022 Health Worker Initiative, which aimed to strengthen US investments in and coordination with strategic partners, including G7 and G20 countries, multilateral and regional organisations, and the private sector, to better protect and advance health workers in the USA and abroad.¹² The Health Worker Initiative can be tied to the climate emergency to help spur global investments to follow. Building the health workforce is a powerful tool for adaptation and resilience and for ensuring a strong public-health response to existing disease burdens.

Safeguarding our domestic and global health in the era of rapid climate change and amid climate policy will require the leadership, integrity, and commitment of US and global leaders. The health community has a fundamental role and responsibility to ensure the success of this mandate. Scientists, clinicians, and academics, in addition to policy makers, can advance the commitment to a human-centred climate response, ensure its accountability and progress, and advance action that addresses the intimate links between climate change and our health.

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US health at home and abroad

Often framed as a product of hard security interests, soft power humanitarian concern, and geopolitical calculation, US global health engagement can seem divorced from health concerns within the nation's borders. Yet health in the USA profoundly shapes the ways in which the country pursues health globally.

The impact of US domestic health on global health starts with which problems the USA takes seriously.

One of the clearest examples is HIV/AIDS. The fight for the US Government to adequately acknowledge and fund an HIV response and later effective HIV treatment in San Francisco, Los Angeles, New York, and across the country created a fierce activist community that together with international counterparts and sympathetic policy makers helped extend access to that same hard-won antiretroviral treatment in sub-Saharan





Africa and other lower-income nations.¹⁻³ Those activists disrupted AIDS conferences, World Trade Organization negotiations in Seattle, and the kick-off event for Al Gore's presidential campaign, unfurling banners that read: "Gore's Greed Kills: AIDS Drugs For Africa".⁴ The heightened attention to global inequities in HIV treatment spilled over to other infectious diseases.⁵ US investment in improving international access to effective medical interventions for malaria, tuberculosis, and paediatric infectious diseases began to increase. The life-saving legacy of these efforts continues today. The US President's Emergency Plan for AIDS Relief (PEPFAR), which was established in 2003, remains the largest commitment by any nation to address a single disease in history. In 2023, the USA was responsible for much of the international development assistance for HIV/AIDS (74.5%, US\$10.5 billion), malaria (39.3%, \$1.3 billion), and tuberculosis (34.6%, \$900.3 million).⁶

US federal public health agencies are a crucial link between domestic and global health. The National Institutes of Health, the Centers for Disease Control and Prevention, and the Food and Drug Administration were created to address domestic needs but now have a central role in many multilateral efforts, from research⁷ to training,^{8,9} to pandemic preparedness and response,¹⁰ to institutional strengthening to standard setting.^{11,12} At times of broad popular support, these agencies expand their global engagement. Conversely, the public loss of confidence as a result of the struggles of these agencies to work together to formulate clear public health guidance and workable diagnostic tests during the COVID-19 pandemic fuelled partisan proposals to split apart or defund agencies.¹³ Proposals to reorganise the civil service and fire many scientists threaten to upend the work of these agencies to prevent and respond to health emergencies at home and abroad.¹⁴

As important as what the USA takes seriously at home is what it does not. The level of US Government support for reproductive health care abroad—epitomised by the so-called Mexico City policy originally implemented by the Reagan administration that blocks funding of organisations that provide abortion services—fluctuates with the political party in the White House.¹⁵ Since the June, 2022, US Supreme Court's decision in *Dobbs v Jackson Women's Health Organization* (hereafter *Dobbs*) upended the constitutional right to choose abortion, millions of people in the USA have lost access

to comprehensive reproductive health care in their home states. An administration that seeks to address women's reproductive needs at home will be supportive of such efforts around the world; an administration that sees reproductive health care in ideological terms is likely to do the same globally. US development assistance for reproductive health is estimated to have increased by 56% and 47%, respectively, during the presidencies of Barack Obama (2009–16) and Joe Biden (2021–23), but it declined by 42% during the presidency of Donald Trump in 2017–20.⁶

A similar but less obvious connection can be found in efforts to address non-communicable diseases. US life expectancy is the lowest among its high-income country peers because of high mortality rates among working-age Americans, due in large part to obesity, diabetes, gun violence, and cardiovascular disease.^{16,17} With prevention of chronic diseases a low priority at home, it is no coincidence that tobacco control, hypertension management, and the other low-cost, effective strategies for reducing non-communicable diseases have also never represented more than 1% of US global health aid, even though those chronic diseases now cause the majority of premature (under age 70 years) deaths in low-income and middle-income countries.^{6,18,19} Similarly, underinvestment in primary care and universal health coverage in the USA limits the nation's leadership in global discussions for supporting access to basic essential health care and financial protection abroad.²⁰

Especially since the COVID-19 pandemic, mistrust in public health officials has complicated efforts to reverse US health declines, even among the most disadvantaged communities. Mistrust in public health is also a phenomenon with associated global health risks. The reluctance of dairy farmers and workers to participate in US public health programmes post-COVID-19 has hindered efforts to abate an outbreak of highly pathogenic avian influenza A(H5N1) and contain the spread of the disease.²¹ Similarly, persistent and widespread distortions and disinformation around vaccines on US social media networks accelerate the spread of diseases around the world.²² When distrust is targeted at global institutions, including WHO, the result is an undermining of sustained US support for the organisation and its negotiation of the proposed Pandemic Treaty.

Health had less of a role in the 2024 US presidential election than it has at any time since the mid-1990s,

when the Clinton administration sought to reform US health care. Candidates mostly avoided the topic of COVID-19. There were no calls to repeal the Affordable Care Act of 2010 or to institute Medicare for All, as there had been in past campaigns. Abortion and reproductive rights loomed large as a US political issue after the *Dobbs* decision, but election debates on the topic were relatively muted after the Trump campaign and the Republican Party, fearful of the backlash from women voters, backed away from calls for a national ban on abortion and adopted a platform that called for “access to birth control, and IVF (fertility treatments)”.²³ Health care rated well below the economy, democracy, and immigration in the top concerns of US voters in 2024.²⁴

This recent relative inattention to health in US domestic politics may have international implications. US global health aid flatlined during the Obama and Trump presidencies from 2011 to 2020 and has been declining after the COVID-19 pandemic.⁶ The latest Biden administration budget request included a \$400 million cut in PEPFAR funding, which would set the programme back to its lowest level of funding since 2018.²⁵ Over the next 6 months, a crowded field of international health and development institutions, from GAVI, the Vaccine Alliance and the Global Fund to Fight AIDS, Tuberculosis and Malaria to the World Bank and WHO, are seeking to raise collectively almost \$50 billion, and their hopes for US fundraising are likely to fall short, irrespective of the US election outcome.²⁶ Many sub-Saharan African governments face mounting debt obligations and exchange rate pressures after the COVID-19 pandemic, limiting their capacity to self-finance health programmes previously funded through development assistance.²⁷ Yet the question of to what degree those aid budgets shrink and what gets prioritised within US global health engagement very much corresponds with the US President’s domestic health priorities. If past is prologue, health at home will matter profoundly for the quest for better health abroad.

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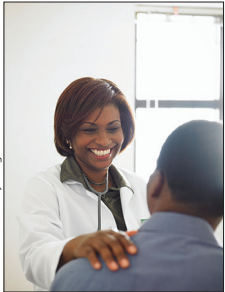
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A public health framework for carceral health



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The USA incarcerates 614 people per 100 000—the fourth highest rate worldwide—at a cost of more than US\$182 billion per year.^{1,2} An estimated 5.5 million people were under the supervision of US adult carceral systems at a single point in time in 2021, which includes people in prison or jail and those on probation or parole.³ 95% of people who are incarcerated are ultimately released and return to their communities.⁴ In 2021, about 450 000 people were released from state and federal prisons, and another 7 million cycled through local jails (remand).^{5,6} Although it is not clear that incarceration reduces crime, it results in a myriad of negative health outcomes for people who are incarcerated, their families and communities, and people working in carceral facilities.^{7–11}

Mass incarceration in the USA has increased over time within a complex historical web of cultural and policy-related factors.¹² Incarceration is both an upstream social determinant of health, affecting rates of infectious and chronic disease, drug overdose, and suicide, and a downstream outcome of other social determinants including racism, poverty, and educational attainment.¹⁰ The impact of incarceration is an urgent public health priority. Here, we present a public health framework to prevent incarceration and mitigate its effects on health, based on concepts of primary, secondary, and tertiary disease prevention.

Primary prevention aims to prevent disease before it occurs by addressing factors that can lead to exposure to, or development of, the disease.¹³ The risk of incarceration can be reduced by addressing upstream social causes of crime and arrest. It will take a multisector approach and strong commitment to systems change to address these issues, which include economic and educational inequities, food insecurity, housing instability, insufficient physical and mental health-care access, and racial bias in the US legal system.^{12,14}

Secondary prevention aims to reduce the impact of disease that has already occurred by detecting and treating it in early stages, preventing recurrence, and ideally returning a person to their original health state.¹³ Increasing access to pathways that are alternatives to

criminal charges or incarceration can reduce the impact of arrest and prevent recidivism. Even with policy and social change, crime is unlikely to entirely disappear. However, criminal charges and confinement are not always necessary; there is evidence that greater benefit can come from rehabilitative diversion programmes that allow people to continue functioning within their communities, without the barriers to housing, employment, and other needs that a criminal record can bring.^{15–17}

Perhaps the most impactful step would be to stop using carceral systems as the primary way to address mental health conditions and substance use disorders. Although jails in particular have become de facto providers of these services, they are not adequately staffed to do so, nor are they well suited to support diagnosis, treatment, and recovery.¹⁸ Instead, sustained investments in community-based mental health and substance use disorder services could prevent contact with the legal system altogether. For people who do make contact, more programmes are needed to divert to these community services.

Tertiary prevention aims to reduce the impact of an ongoing disease that has lasting effects by maximising a person's ability to function, their quality of life, and life expectancy.¹³ Once people are incarcerated, the impact of incarceration on health can be reduced by providing patient-centred health care, supported by public health and oversight systems informed by people with lived experience.

First, the carceral health-care delivery model in the USA should shift from reactive treatment to a proactive prevention model. The current health-care delivery model in most carceral facilities is based on what is known as a sick call system, in which people request health-care services when symptomatic. Although this model can effectively address care needs in a young, healthy population, it cannot safely address the needs of the incarcerated population, which is ageing more rapidly than the general US population and has higher rates of communicable diseases and non-communicable chronic illnesses, including asthma, hypertension, diabetes, mental health conditions, and substance use disorders.^{10,18–20} Shifting to a